

Summary for GP — Aquagenic Pruritus

Prepared from a structured dialog with Claude (Opus 4.8), edited and framed for situational assessment.

Mustafa Uzumeri, PhD — July 17, 2026

Presenting concern: Aquagenic pruritus (AP) with progressive loss of previously stable symptom control.

History

- Long-standing aquagenic pruritus — intense itching on water contact, no rash or wheals.
- **Historically antihistamine-resistant:** prior allergist trial (Atlanta) of high-dose loratadine — up to 4–5× standard tablets per episode, as advised — produced little or no symptom reduction. This resistance is characteristic of AP.
- Controlled for ~5 years on oral beta-alanine taken each morning before showering, with near-complete symptom suppression. Use was literature-informed: published case reports support oral β -alanine for AP (e.g., *Pediatr Dermatol* 2020, PMID 33170524; *Case Rep Dermatol* 2025), and a 2026 *JMIR Dermatology* paper proposes a neuroimmune mechanism (PMID 41880222).
- **Form history:** powder (measured into water) for ~4 years with full control; switched to capsules ($2 \times 750 \text{ mg} = 1500 \text{ mg}$) ~1 year ago with ~8 months of continued full control before symptoms began — arguing against the form change itself as the cause.
- **~3–4 months ago:** intermittent breakthrough itching began; dose increased to 2250 mg ($3 \times 750 \text{ mg}$) in response. Breakthrough episodes have continued despite the higher dose.
- Over the same period, post-dose beta-alanine paresthesia — previously consistent, and a familiar marker of an absorbed/active dose — has become **intermittent**: some doses as strong as ever, others barely noticeable. The reliable protection window (previously ~20 min to 2–3 h post-dose) appears to have shortened.
- **Supply history:** all beta-alanine has been purchased via Amazon. Earlier capsule supplies were from one manufacturer; ~[date from order history] switched to a second manufacturer whose first batch performed slightly better than earlier supplies. Current capsules: **Confident Sports Beta Alanine 750 mg (NPN 80087380** — Health Canada–licensed, NSF-certified facility; **expiry 08/2027)**, a new bottle of that same brand. The licensed, in-date product argues against a gross potency problem, though lot-level variance cannot be fully excluded.
- **Most recent episode (July 17):** woke with itching on a very humid day, consistent with sweat-triggered onset (a known AP pattern). Took the usual 2250 mg after onset — no paresthesia noted, no suppression; note this was mid-attack rather than the usual prophylactic timing. Cetirizine 20 mg taken ~40 min post-shower; itching resolved 1–1.5 h later (unclear whether drug effect or natural resolution — given the historical antihistamine resistance and the typically self-limiting course of attacks, natural resolution is more likely).

- **Net:** a five-year-stable treatment response has degraded over ~3–4 months and did not recover with a 50% dose increase.

Reason for visit

The gradual degradation of a long-stable treatment response, rather than any single attack. Aquagenic pruritus can precede polycythemia vera by years, and worsening itch is a recognized presenting feature.

Requesting

1. **CBC with differential** (serum erythropoietin and JAK2 V617F if indicated by results).
2. **Referral to dermatology** — Women's College Hospital or Sunnybrook — for assessment and management options including narrowband UVB. (*Note: WCH's phototherapy centre, PERC, accepts referrals only from dermatologists, so the referral should be to general dermatology for assessment, with phototherapy as the anticipated pathway.*)

Review of systems — the following are absent

Night sweats, unexplained weight loss, facial flushing/plethora, headache, visual disturbance, left upper quadrant fullness.

Patient-side plan already underway

- Testing fresh beta-alanine **powder** from a verified retail source (brick-and-mortar, CarnoSyn-branded where possible, not sustained-release) — returning to the exact form and regimen that produced the longest stable control, to distinguish product variance from true loss of response.
- Daily log: dose time, food, paresthesia intensity (0–3), exposure, outcome, any antihistamine use.
- Mapping Amazon order dates (bottle boundaries) against the symptom timeline.
- Results available at follow-up.

Medications

Beta-alanine 2250 mg each morning (recently increased from 1500 mg, which was stable for ~5 years). No other medication changes.